

REFINITIV STREETEVENTS

EDITED TRANSCRIPT

MRK.N - Q3 2024 Merck & Co Inc Earnings Call

EVENT DATE/TIME: OCTOBER 31, 2024 / 1:00PM GMT

OVERVIEW:

Company Summary

CORPORATE PARTICIPANTS

Peter Dannenbaum Merck & Co Inc - Senior Vice President, Investor Relations

Robert Davis Merck & Co Inc - Chairman, Chief Executive Officer

Caroline Litchfield Merck & Co Inc - Chief Financial Officer, Executive Vice President

Dean Li Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

CONFERENCE CALL PARTICIPANTS

Trung Huynh UBS - Analyst

Louise Chen Cantor Fitzgerald & Co. - Analyst

Chris Shibutani The Goldman Sachs Group, Inc. - Analyst

Daina Graybosch Leerink Partners LLC - Analyst

Chris Schott JPMorgan Chase & Co. - Analyst

Terence Flynn Morgan Stanley & Co. LLC - Analyst

Carter Gould Barclays Capital, Inc - Analyst

Umer Raffat Evercore ISI - Analyst

Steve Scala TD Cowen - Analyst

Mohit Bansal Wells Fargo Securities, LLC - Analyst

PRESENTATION

Operator

Thank you for standing by. Welcome to the Merck & Co. Q3 sales and earnings conference call. (Operator Instructions) This call is being recorded. If you have any objections, you may disconnect at this time.

I would now like to turn the call over to Mr. Peter Dannenbaum, Senior Vice President, Investor Relations. Sir, you may begin.

Peter Dannenbaum - Merck & Co Inc - Senior Vice President, Investor Relations

Thank you, Shirley, and good morning, everyone. Welcome to Merck's third-quarter 2024 conference call. Speaking on today's call will be Rob Davis, Chairman and Chief Executive Officer; Caroline Litchfield, Chief Financial Officer; and Dr. Dean Li, President of Merck Research Labs.

Before we get started, I'd like to point out that we have items in our GAAP results, such as acquisition-related charges, restructuring costs, and certain other items, and that we have excluded these from our non-GAAP results. There is a reconciliation in our press release.

I will also remind you that some of the statements that we make today may be considered forward-looking statements within the meaning of the Safe Harbor provision of the US Private Securities Litigation Reform Act of 1995.

Such statements are made based on the current beliefs of Merck's management and are subject to significant risks and uncertainties. If our underlying assumptions prove inaccurate or uncertainties materialize, actual results may differ materially from those set forth in the forward-looking statements.

Our SEC filings, including Item 1A in the 2023 10-K, identify certain risk factors and cautionary statements that could cause the company's actual results to differ materially from those projected in any of our forward-looking statements made this morning. Merck undertakes no obligation to publicly update any forward-looking statements.

During today's call, a slide presentation will accompany our speakers' prepared remarks. These slides, along with the earnings release, today's prepared remarks, and our SEC filings, are all posted to the Investor Relations section of Merck's website.

With that, I'd like to turn the call over to Rob.

Robert Davis - Merck & Co Inc - Chairman, Chief Executive Officer

Thanks Peter. Good morning and thank you for joining today's call.

At Merck, we're confronting some of the world's most challenging unmet medical needs with innovative science and differentiated solutions and we're delivering on our key strategic priorities. The strong progress we're making across our business increases the confidence I have in our ability to achieve long-term success and create sustainable value for both patients and shareholders.

We're launching important new products that solve for unmet medical needs, and which have significant commercial potential. We're advancing novel clinical programs across a pipeline of diversified therapeutic areas and modalities. And we're adding to our internal research and development efforts through value-creating business development transactions.

As a result, Merck is moving toward a future with a much more diversified portfolio. Our Phase 3 pipeline has nearly tripled over the past three-plus years to more than twenty unique assets. This will fuel a substantial set of new medicine and vaccine launches over the next five years, in fact, approximately equal to what we launched over the past ten years, and we believe the majority have blockbuster-plus potential.

We have improved long-term visibility and are intensely focused on ensuring strong clinical execution across our diversified set of pipeline opportunities and future growth drivers.

As always, we remain science-driven and patient-focused as we work to bring these medicines and vaccines to those who need them most.

Turning to our third quarter results. Commercial execution is a top priority, and we're highly focused on delivering in the near-term. The strength of our overall portfolio puts us on track to achieve strong full-year growth.

For the quarter, revenues increased 4%, or 7% on a constant currency basis. Our results benefitted from increased usage of KEYTRUDA globally, contributions from new launches, including WINREVAIR and CAPVAXIVE, and strong growth in our Animal Health business. I'm pleased with the launch performance of WINREVAIR, which is consistent with our high expectations, and remain confident in the long-term opportunity it represents for patients and for Merck.

As anticipated, results also reflect a decline in GARDASIL sales year over year. Notably, however, we achieved strong double-digit growth for GARDASIL in almost every major region outside of China.

In China, consistent with the expectations we discussed on our prior earnings call, we shipped less to our commercialization partner, Zhifei, and we expect fourth quarter shipments will be at a similar level to the third quarter. Overall channel inventories of GARDASIL have decreased, which is directionally encouraging, while inventory at Zhifei remains above historical levels.

We are highly focused on this market and are making progress with Zhifei to increase promotional resources and patient education efforts. We expect these efforts to translate to increased patient activation and demand, but as we've said, this will take time.

Taking a step back, we're proud of the role that GARDASIL is playing in helping prevent certain HPV-related cancers. There is a wide range of long-term growth opportunities around the world due to the tremendous remaining need to protect more individuals, with less than 10% of the global eligible population vaccinated and meaningful opportunities to improve vaccination completion rates, gender-neutral vaccination rates, mid-adult coverage and access in low- and middle-income markets.

This includes in China, where there is an attractive long-term opportunity given the significant number of females yet to be immunized, and the potential approval for males next year. We're highly focused on using our scale and strong capabilities to drive education and awareness of the benefits of HPV vaccination and to reach and protect more patients globally. As such, we remain confident in our goal of achieving greater than \$11 billion of sales by 2030.

Turning to our research efforts, we achieved important clinical and business development milestones this quarter. Starting with our clinical advancements, in vaccines, we presented positive Phase 2b/3 results for clesrovimab, our investigational monoclonal antibody for the prevention of RSV in infants. Based on its differentiated profile and robust clinical data, we believe this opportunity is underappreciated. We're moving swiftly to make this important prevention option available in the US during the 2025-26 RSV season.

In pneumococcal, ACIP voted to expand vaccination recommendations for CAPVAXIVE to include adults ages 50 to 64. We're pleased to bring CAPVAXIVE's effectiveness in protecting against invasive pneumococcal disease to this new cohort of patients, and this recommendation reinforces our confidence in its blockbuster commercial potential.

In oncology, it's been ten years since KEYTRUDA's first US approval, and we remain highly focused on advancing standard of care and maintaining durable leadership in oncology. At ESMO, we presented overall survival and long term follow up data for KEYTRUDA, and data also were shared from our innovative ADCs, underscoring our commitment to building a broad and diverse oncology pipeline.

In infectious diseases, we shared Phase 2 data from our collaboration with Gilead for the combination of once-weekly oral islatravir and lenacapavir for HIV treatment in treatment experienced adults. This data represents just a portion of our broad HIV development efforts to further advance the field.

In immunology, we shared positive Phase 2 maintenance data for our investigational TL1A candidate tulisokibart in ulcerative colitis and Crohn's disease that adds to positive prior clinical results.

Both our HIV and immunology programs address patient populations with significant remaining unmet need despite available therapies. Our investments in these therapeutic areas reflect our belief in the opportunity to deliver additional value to patients and fortify our sustainable innovation engine for the long-term.

Regarding business development, we continued to leverage our scientific expertise to identify promising therapeutic targets that can further broaden and fortify our pipeline. This quarter, we acquired a novel investigational bispecific T-cell engager from Curon with potential applications in both oncology and immunology, complementing our efforts in each of these areas of high unmet need and we closed the acquisitions of EyeBio and the Elanco aqua business.

In summary, we remain highly focused on strong commercial and operational execution to enable access to our medicines and vaccines. We're also continuing to make strategic investments in our pipeline that we're confident will lead to important innovations for patients and future growth drivers for our company.

I want to again recognize the commitment and efforts of our team across the world. Based on our continued progress, I'm more confident in our longer-term future today than I was a year ago, and I believe that Merck is even better positioned to deliver value to patients, shareholders and to all of our stakeholders long into the future.

With that, I'll turn the call over to Caroline.

Caroline Litchfield - Merck & Co Inc - Chief Financial Officer, Executive Vice President

Thank you, Rob. Good morning.

As Rob highlighted, we delivered another strong quarter. The fundamentals of our business remain healthy, fueled by robust global demand for our innovative portfolio. This strong performance reinforces the conviction we have in our science-led strategy and in our outlook for continued growth.

We remain confident in our ability to consistently deliver strong results in the short-term, while we make disciplined investments in leading-edge science, which positions us to generate lasting value for patients, customers, and shareholders.

Now, turning to our third quarter results. Total company revenues were \$16.7 billion, an increase of 4%, or 7% excluding the impact of foreign exchange. The following revenue comments will be on an ex-exchange basis. Our Human Health business sustained its momentum with sales increasing 8% primarily driven by Oncology. Our Animal Health business also delivered strong performance, with sales growth of 11%.

Turning to the performance of our key brands. In Oncology, sales of KEYTRUDA grew 21% to \$7.4 billion. Global growth was driven by increased uptake from earlier stage cancers and continued robust global demand from metastatic indications.

In the US, KEYTRUDA grew across a broad range of tumor types. In the earlier-stage setting, the largest driver was increased use in resectable non-small cell lung cancer. KEYTRUDA's market leadership continues to grow as part of a treatment regimen in the perioperative setting, building on its existing leadership position as adjuvant therapy.

In metastatic disease, we saw increased uptake in first-line advanced urothelial cancer based on KEYNOTE-A39. KEYTRUDA plus Padcev continues its leadership in new patient starts, outpacing platinum chemotherapy-based regimens.

Outside the US, KEYTRUDA growth was driven by increased uptake in earlier stage cancers, including high risk, early-stage triple negative breast cancer, as well as continued demand from patients with metastatic disease. Inflation related price increases consistent with market practice in Argentina also contributed to growth.

Lynparza alliance revenue grew 13% driven by increased global demand. Lenvima alliance revenue declined 4% due to the timing of shipments last year. WELIREG sales more than doubled to \$139 million driven by increased uptake in certain patients with previously treated advanced renal cell carcinoma in the US.

In Vaccines, GARDASIL sales were \$2.3 billion, a decrease of 10%, driven by a decline in China. In the US, sales benefitted from CDC purchasing patterns, as well as price and demand. Outside the US, sales increased by double digits in almost every region driven by robust demand.

In pneumococcal, VAXNEUVANCE sales increased 13% to \$239 million, driven by ongoing launches in international markets. We are also excited by the recent launch of CAPVAXIVE, which is off to an encouraging start.

In Cardiovascular, the launch of WINREVAIR continues to gain momentum, with global sales of \$149 million.

In the US, we have seen steady progress in adding new patients. During the quarter, approximately 1,700 new patients received a prescription, bringing the total number of new patient prescriptions to more than 3,700 since launch.

Based on our experience to date, approximately 80% of those patients will receive commercial product. Given this, and the approximate one-month timeframe to complete the steps to commence therapy, more than 2,600 new commercial patients have started treatment since launch.

We are seeing physicians prescribe WINREVAIR to more of their patients and new physicians prescribe the product. Through the end of September, nearly 800 physicians have written at least one prescription, with most prescribers coming from either large academic centers or larger private practices. Physicians are continuing to prioritize the sickest patients, who have already been receiving multiple PAH therapies.

We are also making important progress in enabling access. We have achieved coverage for approximately 60% of lives, nearly doubling the amount from last quarter. Many payors have established coverage policies consistent with the label or STELLAR study criteria.

Outside the US, initial feedback from scientific leaders has been positive following the recent EU approval. We are pleased that the first patients in Germany have received a prescription for WINREVAIR. We look forward to securing reimbursement in other European countries, which typically takes twelve months, as well as launching in other international markets.

In summary, we are excited with the continued progress of the launch and look forward to positively impacting more patients with pulmonary arterial hypertension. Our Animal Health business delivered strong growth, with sales increasing 11%.

Companion animal sales grew 17%, driven by uptake from new product launches, including the long-acting BRAVECTO injectable in certain international markets, as well as price. Livestock sales grew 7%, reflecting higher demand for poultry and swine products, the inclusion of sales from the recently acquired aqua portfolio from Elanco, which closed in mid-July, and price.

I will now walk you through the remainder of our P&L, and my comments will be on a non-GAAP basis. Gross margin was 80.5%, an increase of 3.5 percentage points driven by reduced royalty rates for KEYTRUDA and GARDASIL as well as favorable product mix.

Operating expenses increased to \$8.5 billion, including \$2.2 billion of charges related to the acquisition of EyeBio and a promising candidate from Curon. Excluding these charges, operating expenses grew 9%, reflecting strategic investments in support of our innovative early- and late-phase pipeline and key growth drivers.

Other income was \$193 million, which includes a payment of \$170 million from Daiichi Sankyo related to the collaboration for our T-cell engager. Our tax rate of 21.9% includes an unfavorable impact from the EyeBio and Curon transactions. Taken together, earnings per share were \$1.57.

Now turning to our 2024 non-GAAP guidance. The continued operational strength of our business has enabled us to narrow our full-year revenue guidance. We now expect revenue to be between \$63.6 and \$64.1 billion. This guidance range represents strong year over year growth of 6% to 7%, including an approximate 3 percentage point negative impact from foreign exchange using mid-October rates.

Our gross margin assumption remains approximately 81%. We expect operating expenses to be between \$27.8 and \$28.3 billion, which now includes the \$750 million one-time charge related to the asset acquisition from Curon. As a reminder, our guidance does not assume additional significant potential business development transactions.

Other expense is now expected to be approximately \$100 million, including the benefit of the \$170 million payment from Daiichi. Our full-year tax rate is expected to be between 16.0% and 17.0%, which includes an unfavorable impact related to the Curon transaction. We assume approximately 2.54 billion shares outstanding.

Taken together, we expect EPS of \$7.72 to \$7.77. This range includes a negative impact from foreign exchange of approximately [\$0.30], using mid-October rates.

Recall our prior guidance range was \$7.94 to \$8.04. Including the one-time charge of \$750 million, or \$0.29 per share, related to the asset acquisition from Curon and the \$170 million, or \$0.05 per share, benefit from the payment from Daiichi, our prior guidance range would have been \$7.70 to \$7.80, with a midpoint of \$7.75. Therefore, our current guidance midpoint is unchanged.

Now turning to capital allocation, where our strategy remains the same. We will prioritize investments in our business to drive near- and long-term growth. We will continue to invest in our key growth drivers and expansive pipeline of novel candidates, each of which has significant potential to address important unmet medical needs.

We remain committed to our dividend, with the goal of continuing to increase it over time. Adding compelling science through business development remains a priority and we are well positioned to pursue additional value-enhancing transactions. We will continue to execute a modest level of share repurchase.

To conclude, as we finish the year, we are confident in the momentum of our business, underpinned by robust global demand for our innovative medicines and vaccines. Our unwavering dedication to leverage compelling science to save and improve the lives of the patients we serve has put us in a position of financial and operational strength.

Our commitment to bring forward important innovation will enable us to deliver value to patients, customers and shareholders well into the future.

With that, I'd now like to turn the call over to Dean.

Dean Li - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

Thank you, Caroline.

We have continued to execute on our strategy of diversifying in oncology and expanding into new therapeutic areas, while also investing in novel modalities and technologies. The third quarter was marked by several important clinical and regulatory milestones.

I will first cover infectious diseases on the broader portfolio. As Rob noted, last week ACIP voted to update the adult age-based pneumococcal vaccination guidelines and recommend the use of CAPVAXIVE, our 21 valent pneumococcal conjugate vaccine, for individuals 50 years and older.

CAPVAXIVE is specifically designed to cover serotypes responsible for the majority of invasive pneumococcal disease and includes serotypes responsible for approximately 84% of cases of IPD in adults 50 and older, based on national level CDC data from 2018 to 2022.

Building on our company's proud legacy in infectious diseases, detailed findings from a pivotal Phase 2b/3 clinical trial evaluating clesrovimab, our investigational respiratory syncytial virus preventative monoclonal antibody for the protection of infants entering their first RSV season, were presented at IDWeek.

Clesrovimab, administered as a single-dose immunization to healthy pre-term and full-term infants, regardless of weight, met all trial endpoints, with consistent results through both five-month and six-month time points.

The incidence of adverse events and serious AEs were comparable between treatment and placebo groups, with no treatment or RSV-related deaths. As shown in the summary tables in the data press release, clesrovimab immunization is being studied across mild, moderate, and severe RSV disease endpoints.

Importantly, clesrovimab significantly reduced the incidence of RSV, the primary endpoint, and hospitalizations associated with RSV infection through five months by more than 84%, the secondary endpoint. It was also observed that clesrovimab reduced RSV lower respiratory infections by more than 90%, through five months, the tertiary endpoint.

Positive results were also presented from an interim analysis in a separate Phase 3 study, evaluating the safety and efficacy of clesrovimab versus palivizumab in infants and children at increased risk for severe RSV disease.

If approved, clesrovimab would be the first and only immunization designed to provide infants with direct, rapid and durable protection for the full six-month RSV season with the convenience of one dose, regardless of weight or month of birth.

Also, at IDWeek, positive data were presented from the Phase 2 study of the combination of islatravir and lenacapavir, Gilead's HIV-1 capsid inhibitor, as a once-weekly oral treatment option for people living with HIV. The 48-Week results build on the positive 24-Week data previously presented. These findings reinforce the strength of our HIV pipeline as we evaluate multiple, promising candidates in preventative and treatment settings.

Next to oncology, KEYTRUDA continues to raise the bar in treating earlier stages of disease. In the earlier stage setting, KEYTRUDA is the only PD-1 or PD-L1 to date to receive FDA approvals for nine indications and is the only one to demonstrate a significant overall survival benefit in four earlier stage settings: non-small cell lung cancer, renal cell carcinoma, cervical cancer, and triple negative breast cancer. And as Caroline spoke to, we continue to see strong patient impact.

KEYTRUDA based regimens have demonstrated positive results across thirteen pivotal trials in eight tumor types. We look forward to building on the data in the earlier stage setting. We recently announced positive topline results for the KEYNOTE-689 trial, evaluating KEYTRUDA as a perioperative treatment for patients newly diagnosed with stage III or IVA, resected, locally advanced head and neck squamous cell carcinoma.

The trial met its primary endpoint of event-free survival and demonstrated an improvement in major pathological response, a key secondary endpoint. This is the first positive trial to show a statistically significant benefit of neoadjuvant plus adjuvant anti-PD-1 treatment for newly diagnosed patients with resected locally advanced head and neck squamous cell carcinoma in twenty years.

Results will be submitted to regulatory authorities and, if approved, would mark the tenth indication of a KEYTRUDA based regimen for the treatment of earlier stage cancer.

On the regulatory front, we received FDA approval for KEYTRUDA in combination with pemetrexed and platinum chemotherapy, for the first-line treatment of adult patients with unresectable advanced or metastatic malignant pleural mesothelioma based on results from the KEYNOTE-483 study. This brings the number of distinct FDA approved indications for KEYTRUDA to 41.

The European Commission approved three KEYTRUDA based regimens which include: in combination with Pfizer's Padcev, a nectin 4-targeting antibody drug conjugate, for the first-line treatment of adult patients with unresectable or metastatic urothelial carcinoma based on the KEYNOTE-A39 study; in combination with carboplatin and paclitaxel, for the first-line treatment of primary advanced or recurrent endometrial carcinoma and; in combination with chemoradiotherapy, for the treatment of FIGO 2014 Stage III-IVA locally advanced cervical cancer in adults who have not received prior definitive therapy based on KEYNOTE-868.

KEYTRUDA was also approved by the Japanese Ministry of Health for three indications: in combination with Pfizer's Padcev, a nectin 4-targeting antibody drug conjugate, for the first-line treatment of adult patients with radically unresectable urothelial carcinoma based on the KEYNOTE-A39 study; for patients with radically unresectable urothelial carcinoma who are not eligible for any platinum-containing chemotherapy based on KEYNOTE-052; as well as in combination with chemotherapy as neoadjuvant treatment, then continued as monotherapy as an adjuvant therapy, for patients with non-small cell lung cancer based on findings from KEYNOTE-671.

At the European Society for Medical Oncology Congress, three presentations for KEYTRUDA were showcased during Presidential Symposium sessions. These include: overall survival data from the KEYNOTE-522 trial in high-risk early-stage triple-negative breast cancer; overall survival data from the KEYNOTE-A18 trial in patients with newly diagnosed, high-risk, locally advanced cervical cancer, and 10-year follow-up overall survival data from the KEYNOTE-006 trial showing the long-term benefit over ipilimumab in patients with advanced melanoma.

And finally in oncology, we continue to advance our increasingly diverse pipeline including our efforts to evaluate the potential of new combination regimens to improve patient outcomes.

Earlier this month, we announced a clinical development collaboration with Exelixis for their investigational tyrosine kinase inhibitor, zanzalintinib which will be evaluated in combination with: KEYTRUDA, for the treatment of patients with head and neck squamous cell carcinoma, and with WELIREG, for the treatment of patients with renal cell carcinoma.

In collaboration with Daiichi Sankyo, we initiated IDEate-Lung02, a Phase 3 trial, evaluating ifinatamab deruxtecan, a B7-H3 directed antibody drug conjugate, for the treatment of patients with relapsed small cell lung cancer. We also expanded our agreement to evaluate the combination of I-DXd with MK-6070, an investigational delta-like ligand 3 targeting T-cell engager. Evidence from clinical studies for each candidate provides strong rationale for evaluating this combination regimen.

Next to our broader portfolio. As Caroline mentioned, we were pleased to receive approval from the European Commission for WINREVAIR, expanding the reach of this treatment option, which we believe has the potential to transform the treatment journey for patients suffering from pulmonary arterial hypertension.

In immunology, 50-week efficacy and safety data for tulisokibart, our investigational humanized monoclonal antibody directed to tumor necrosis factor-like cytokine 1A, from the Phase 2 ARTEMIS-UC and APOLLO-CD studies in ulcerative colitis and Crohn's disease were presented at the United European Gastroenterology Week Congress. The results reinforce the potential of tulisokibart to help patients achieve long-term clinical remission. The Phase 3 trials continue to actively enroll patients.

Finally, in ophthalmology, soon after completing our acquisition of EyeBio, we initiated the Phase 2b/3 BRUNELLO trial for MK-3000, an investigational tetraivalent, tri-specific antibody that acts as an agonist of the Wingless-related integration site signaling pathway, being evaluated for the treatment of diabetic macular edema.

We are executing on our ONE PIPELINE approach, augmenting and complementing our internal programs through business development.

Recently, we completed the acquisition of CN201 a novel CD3xCD19 T-cell engager now known as MK-1045, from Curon Biopharmaceutical. MK-1045 has shown to significantly deplete B cell levels with potential applications in B-cell malignancies and autoimmune diseases. As with our previous acquisitions, we plan to seamlessly advance clinical development with rigor and speed.

In closing, during the quarter we saw a regular cadence of late-phase pipeline advancements including regulatory milestones and data readouts. We continue to make progress across therapeutic areas, and I look forward to providing further updates on our programs in 2025.

And now, I turn the call back to Peter.

Peter Dannenbaum - Merck & Co Inc - Senior Vice President, Investor Relations

Shirley, we're now ready for Q&A. (Event Instructions)

QUESTIONS AND ANSWERS

Operator

(Operator Instructions) Trung Huynh, UBS.

Trung Huynh - UBS - Analyst

Just on GARDASIL, given the inventory levels remain elevated, you've noted this declining demand. How should we think about dynamics as we head into 2025, given that increase in promotional activity, but that's balanced by the inventory work down? Will you take time? Or do you have any color on when we'll see an inflection to that returning growth? Thank you.

Robert Davis - Merck & Co Inc - Chairman, Chief Executive Officer

Great. Thanks for the question. Obviously, we're very focused on GARDASIL in China. But maybe just to step back for a second, we continue to be very proud of the contribution that GARDASIL is making for patients and people around the world to really address and hopefully eliminate long-term cervical cancer as well as other HPV-related cancers. So that important work will continue.

And importantly, as we commented in our prepared remarks, while China did decline, and I'll speak to China in a second, overall, we saw strong double-digit growth in really nearly every other region around the world, which is showing the progress we're making, and which is why we continue to have such confidence in the long-term potential for this.

But as it relates to China specifically, and as we think about 2025, I don't want to give specific guidance because obviously, we're still working through our 2025 plan. But what I would say is we do expect to continue to see a decline in shipments into China into 2025. And as we had highlighted before, this is happening a little bit earlier than we originally expected.

But we had always expected that over time, as we work through the bolus, we would see the female opportunity decline and then hopefully seeing growth come with bringing the male opportunity, which we would expect to see with approval, assuming it comes next year. So that's how we see it progressing.

So as we think about 2025, we see China really in the \$2 billion to \$3 billion range as far as an opportunity for 2025 and for the next several years with the opportunity in males really being the growth driver. And at that level, we would expect for overall Merck that you're going to continue to see, based on the portfolio we have, solid growth. So I think that's just important to kind of frame where we're seeing things but understanding we're focused on this, we're bringing our efforts to drive demand and we're going to make progress. We are making progress, but it's going to take some time.

Operator

Vamil Divan, Guggenheim Securities.

Unidentified Participant

This is Edouard on for Vamil from Guggenheim. Just on WINREVIR, what are you seeing in terms of safety since it's been rolled out commercially? Any sense of how many patients are on the low dose versus the high dose? And any estimate of what percent of patients need to have their dose reduced or stopped due to the safety issues? Thank you.

Robert Davis - Merck & Co Inc - Chairman, Chief Executive Officer

Yes. I'll maybe let Dean start and then I can jump in.

Dean Li - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

From the safety issue, we haven't had any profound or important signals at all in relationship to the patients that we've been serving. So that has not been an issue to date in the patients that have gotten WINREVAIR. But more broadly?

Robert Davis - Merck & Co Inc - Chairman, Chief Executive Officer

Yeah. So just to give you a sense of what we're seeing in the marketplace. As we said in the prepared comments, we are seeing the sickest patients being predominantly about 80% of the patients we have today. So those would be people on triple therapy. And as we're looking at the kits being used, we're still predominantly seeing use of single vials, not double vials.

As we're still bringing a lot of people on to therapy, remember that when a patient starts therapy, they start on a single vial and then they would transition either to maintaining on single or going to double depending on their weight over time.

So we expect to see, as we saw with STELLAR, that you would end up with about two-thirds of patients would be on the single vial and about one-third of patients on the double vial. Right now, that's a little bit more skewed towards single, but we expect it still to get there over time.

And importantly, on your broader question, Dean gave one sense about where we see safety. I think the best real-world statement around safety, we're seeing very strong adherence and really seeing less discontinuations than we expected. So the anecdotal information we're getting from the marketplace as far as the real-world experience, I would say as said, that safety is not an issue.

Operator

Louise Chen, Cantor.

Louise Chen - Cantor Fitzgerald & Co. - Analyst

I wanted to ask you about your outlook for 2025, what are the major puts and takes are? And when do you plan to give that guidance?

Robert Davis - Merck & Co Inc - Chairman, Chief Executive Officer

Yes. I'll turn it over to Caroline for that.

Caroline Litchfield - Merck & Co Inc - Chief Financial Officer, Executive Vice President

So Louise, we plan to give our guidance with the fourth quarter earnings, as we ordinarily do. But if I give you a sense of the overall pushes and pulls, if I start with 2024, it's been a really strong year for our company. As we've noted, we expect to deliver 6% to 7% growth, while we're investing in and really driving our pipeline and have delivered strong underlying operating margin improvement.

As we look to 2025, we expect solid growth for our business. And the key opportunities that we have is the continued impact for KEYTRUDA, especially in earlier-stage cancers, but the tumor types we would call out would be early-stage non-small cell lung cancer. We've got great opportunity in women's cancers, including early-stage triple-negative breast, but also endometrial and cervical cancer. And of course, bladder cancer is a real opportunity for growth.

We are confident where we are with WELIREG and the opportunities there to continue to see strong performance and impact for patients. And of course, we have the recent and upcoming launches that includes WINREVAIR, where we have high expectations for growth; it includes CAPVAXIVE, where we expect to get a majority share over time; and it also includes the opportunity for the launch of clesrovimab in RSV ahead of the '25-'26 season. And as we noted, Animal Health is a strong contributor to growth, and we expect that to continue into next year.

We have a few headwinds. One is the expiration of our agreement with J&J for the immunology products, REMICADE and SIMPONI. And as Rob noted, we do expect sales of GARDASIL in China to decline in 2025 compared with 2024. However, all of that said, we're very confident in the opportunity to drive solid growth for our business in '25, while we invest fully behind our expansive pipeline, invest in the commercial activities to ensure excellence in the marketplace and expect to deliver operating margin expansion.

Operator

Chris Shibutani, Goldman Sachs.

Chris Shibutani - *The Goldman Sachs Group, Inc. - Analyst*

The capital allocation opportunity is something that you have been very consistent about providing us updates. Given some of the push/pulls in the business, in particular vulnerability with GARDASIL, I think you have capacity and you have also mentioned interest in certain areas, I would note, with obesity. Can you update us on where you are with your appetite, scale, therapeutic area? Anything that would give us insight into your ability to use that as a lever to help perhaps offset some of this uncertainty tied to GARDASIL.

Robert Davis - *Merck & Co Inc - Chairman, Chief Executive Officer*

Yeah. No, Chris, thanks for the question. Obviously, I feel very good about the business development we've done, including some of the transactions that Dean just highlighted in the prepared remarks, and we're going to continue to look to do more. So while we feel good about where we are and actually, as I made a comment in the prepared remarks, we have more Phase III assets now than we had -- we've more than tripled it from where we were three years ago with more than 20.

So we have a lot there, but we need to add more. And we're looking across all therapeutic areas. Our approach remains where it's always been, which is we start with the science and where we see a scientific opportunity that addresses an unmet need and where we think Merck can strategically play. If we see that aligned with value, we will act.

As it relates to obesity, our goal and strategy there continues to be focusing on the second- and third-generation opportunities in that space. We've talked in the past, thinking more of a focus on oral opportunities and those that can be in combination focused on outcomes that go beyond just obesity but to actually medical outcomes. So we continue to look but we're going to be disciplined. And if we don't see something where we think it makes sense from a value perspective and a strategy, we won't move.

Beyond that, I would say, you should continue to see us look in areas of oncology, immunology and broader cardiometabolic. All the places where we're playing today are areas where we continue to look for opportunities down the road. From a capacity perspective, just to be clear, we continue to think we have capacity, frankly, to do pretty much anything of any size. But our focus area continues to be mainly in that \$1 billion to \$15 billion range.

Operator

Daina Graybosch, Leerink Partners.

Daina Graybosch - *Leerink Partners LLC - Analyst*

Yeah. Can you talk a little bit more about the pneumococcal opportunity? What do you think is the incremental market size from the expansion down to age 50? And is there anything you're looking for in the MMWR language when they publish that recommendation that would drive more or less share to CAPVAXIVE versus the competitor?

Robert Davis - *Merck & Co Inc - Chairman, Chief Executive Officer*

Yeah. Maybe Dean, do you want to start and we can jump in?

Dean Li - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

Yes. I mean I think it was interesting to watch the ACIP. They're balancing how to make the best recommendations. And we should just remember, we have a 21-valent vaccine that gives 85% adult protection. That was very clear. And there is a competitor who has a 20-valent that has a 54% adult protection. And I think that was very clear at the ACIP. And I would just remind myself that previously, when they thought about doing the 50 to 64 just with the 20-valent, there was a decision not to proceed.

And I really do think the health economic data and the data that we show for the 21-valent, given it's 85% versus 54% is what drove them to finally move and address a really important need, which is the health equity issues in relationship to a pneumococcal vaccination by lowering it to 50 to 64 and lowering it for everyone. But in terms of the dynamics in the market.

Caroline Litchfield - Merck & Co Inc - Chief Financial Officer, Executive Vice President

So in terms of the commercial opportunity we have, given the strength of the data that Dean has just shared, we feel very confident in our opportunity. And to remind in terms of the number of patients in the United States, there's around 120 million people over the age of 50.

60 million of those are in the 50 to 64 age group, which now given the ACIP recommendation, provides a tremendous opportunity for us to activate that patient cohort to go and get this vaccine as well as the group that sit above the age of 65 given the strength of our data. So we are ready. We have supply, and we're looking forward to ensuring that we can protect as many people as possible.

Operator

Chris Schott, JPMorgan.

Chris Schott - JPMorgan Chase & Co. - Analyst

Just another one on GARDASIL. Rob, I believe you said kind of \$2 billion to \$3 billion per year opportunity for China over the next few years. Just a couple of quick ones there. First, what does that compare to where China is going to shake out for Merck this year? And is that \$2 billion to \$3 billion number what was reflected in the \$11 billion longer-term target?

Robert Davis - Merck & Co Inc - Chairman, Chief Executive Officer

Yeah. So to give you just a sense of where we are, and we don't normally give product-level guidance or specifics on a quarterly basis. But given the importance and focus on GARDASIL, just to give you a sense of where we were with China GARDASIL in the third quarter, it's about approximately \$500 million in the third quarter.

And as we said, we would expect to ship about the same amount in the fourth quarter. So you should expect that the fourth quarter itself would also be in that \$500 million range. And so that kind of gives you a sense of where we are.

So as you look forward to 2025, obviously, as we think in 2025 and over the next several years, if you're running in that \$2 billion to \$3 billion range, that's why we made the comment that with that and given the other opportunities we see around the world, we remain confident in our ability to get to the \$11 billion by 2030.

So we are contemplating that \$2 billion to \$3 billion over the next several years in China as the contribution that it would make to get us to where we need. Understanding also that long term, we do expect to be able to have the potential for growth driven by the male opportunity in China and then, obviously, continuing to drive more broadly around the rest of the world.

Operator

Terence Flynn, Morgan Stanley.

Terence Flynn - Morgan Stanley & Co. LLC - Analyst

Great. Maybe just one question and one clarification. Just on the GARDASIL side, Rob, that's the \$2 billion to \$3 billion includes the males or it does not include the males? Sorry, I was a little unclear based on the last response. And then obviously, next-gen IO remains a big focus for both Merck and the industry.

I know, Dean, you talked a lot about some of the efforts Merck has made on this front. Would just be curious to get your perspective on the PD-1, PD-L1 VEGF bispecifics. Obviously, there's some data emerging out of China there. Just any thoughts on that data and where that might fit from a competitive landscape perspective or if that's a target Merck has considered at all?

Robert Davis - Merck & Co Inc - Chairman, Chief Executive Officer

Yeah. So I'll take the first part of that question, Terence. So the answer is yes. the \$2 billion to \$3 billion over the next several years does include male, but we have the opportunity as you look longer term to drive growth with that opportunity. That was really the point we were trying to make.

Dean Li - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

In relation to your question in relationship to the vascular endothelial growth factor pathway, it is an important axis. I just -- there's been a lot of data in that. It's not just the most recent data. Merck has made a substantial investment in that pathway. Our collaborations with Eisai and Exelixis is along that pathway. It's PD-1 in this case with the VEGF RTK inhibitors. Other companies have taken their PD-L1 and added to, for example, Avastin. And I would just emphasize, our interest in HIF-2 alpha is also related to vascular endothelial growth factor.

In relationship to the specific question about Summit, that deep expertise that has developed in that VEGF, I think it's really important for all of us to watch carefully, and we are watching carefully the ability to turn PFS into OS. The other issue is that in China, the health system is a little bit different as to how they treat individuals, but also the patients here are also different in terms of their responsiveness to VEGF.

I would also emphasize that the experiment that was done in China is an experiment that we would be unable to do, for example, in the United States. We would not be able to compare a new combo against KEYTRUDA monotherapy in the PD-L1 1% through 49% PD-L1. That's something we would not be able to do because in that situation, the comparator is not monotherapy, but it's 189, KEYNOTE-189.

But broadly speaking, combos will be important, combos to deepen it, to go earlier stage will be important. I think vascular endothelial growth factor pathways are important, INT will be important, RAS will be important, ADCs will be important. And all of those readouts will come, some before 2028, some around 2028 and some will happen after 2028.

Operator

Carter Gould, Barclays.

Carter Gould - *Barclays Capital, Inc - Analyst*

Appreciate all the GARDASIL color. Maybe on WINREVAIR, can you comment on the impact of stocking in the quarter? And I'm trying to also kind of square, Rob, some of your comments around sort of the efforts to work through those sickest patients. Kind of maybe help frame for us kind of how far you're along sort of that initial bolus of patients, those later-line patients within the STELLAR label.

Caroline Litchfield - *Merck & Co Inc - Chief Financial Officer, Executive Vice President*

So Carter, in the quarter, over 80% of the revenue is to support demand. And so a very small level of inventory build, and we'd expect inventory dollars to build over time given the growth that we expect in the product. In terms of the patients that are being prescribed the product, as Rob noted, a large portion, it's actually about 80% of the patients are on background triple therapy or in combination with the prostacyclin. So we're making really good progress in that segment.

At the same time, a large number of the physicians have prescribed WINREVAIR now to patients that are not on a prostacyclin background therapy, which gives us confidence for the ability to move to the patients who are more dual therapy over time. And we remain confident in the opportunity in line with our high expectations here to benefit patients and to drive significant commercial benefit for our company.

Operator

Umer Raffat, Evercore ISI.

Umer Raffat - *Evercore ISI - Analyst*

I have -- I wanted to focus on GARDASIL just a little more. That was very helpful commentary, Rob. You mentioned \$500 million in sales in 3Q and 4Q to China, which obviously is \$2 billion run rate. My question is, was that shipping to demand?

Because if so, what that means is inventory sitting at your bay is still something that would need to be worked down in 2025 as well as possible 9-valent generic entry -- sorry, 9-valent local competition entry in 2025 as well. How do you factor those 2 dynamics into thinking about the 2025 number of \$2 billion to \$3 billion in China?

Robert Davis - *Merck & Co Inc - Chairman, Chief Executive Officer*

Yeah. So if you look at what's happening in the overall marketplace and just to give you a sense, from an inventory perspective, maybe starting there. If you look at overall inventory levels in China, they did come down. And that's taking into account, and this is for GARDASIL. GARDASIL at Zhifei, which frankly remains high and grew slightly, but that was more than offset by reductions in the CDCs and the points of vaccination.

So that's a good sign that we're seeing overall inventories coming down, which also would point to the fact that as we're looking at demand, which we're seeing stabilize, we think we're at a position now whereas we're starting to talk about what we're shipping. Our expectation is we are shipping below demand.

So we have been working very constructively with Zhifei to think about this, both as what we're doing in this year is, frankly, as well as we're continuing to have constructive dialogue around 2025. Our intention would be to balance the need to get product into the marketplace to meet the demand at the same time, allowing for Zhifei to bring down their inventories over time.

So we're very thoughtful on how we're thinking about it, and we've done that, taking into account our expectations of both the female competitive launch that could come next year, but also the opportunity that a male approval early next year could allow us to have. So all of those factors are in as we think about that \$2 billion to \$3 billion number.

Operator

Steve Scala, TD Cowen.

Steve Scala - TD Cowen - Analyst

I mean all things considered, is it still possible to see global GARDASIL growth in 2025? And Rob, you noted at the start that recovery in China will take time, but it sounds like you have good visibility now because you're giving this \$2 billion to \$3 billion guidance for GARDASIL in China. So I'm unclear what it is that we're waiting for.

Robert Davis - Merck & Co Inc - Chairman, Chief Executive Officer

Well, what we're talking about taking time is basically to work down the inventory and to then build demand over time so that we can continue to drive that market. What we're giving you is kind of what we see as the baseline of China.

Our hope is that we'll do better. And we're going to put the work in to do better and to continue to drive long term. I think that what I'm trying to make sure everyone hears is this isn't going to be solved next quarter. It's going to take us through probably 2025, but we're thoughtful on how we're doing it. We're working with Zhifei in a constructive manner to do it.

So those are the elements are going to take time because we need to build the demand. We know the opportunity is there with 120 million females still out there to go after and with potentially 200 million males with the male opportunity. We have to activate that demand to make sure we can drive that business. So that's really what we're focusing on.

As far as it relates to GARDASIL for 2025, I don't think we really want to get into giving product line guidance right now. We were very specific to China because of the concerns that were there. And I wanted to make sure you know that we see solid overall growth for Merck because that's important to have context. But beyond that, we normally wouldn't be giving guidance at this point in time.

Operator

Mohit Bansal, Wells Fargo.

Mohit Bansal - Wells Fargo Securities, LLC - Analyst

Great. My question is regarding V90 -- V940 in lung cancer. I see that you have started Phase III trial. Could you please help us understand what kind of data you have seen in earlier stages, which makes you move into Phase III? I don't think we have seen a lot of data from our side here.

Dean Li - Merck & Co Inc - Executive Vice President, President - Merck Research Laboratories

Yeah. So this is Dean. I would just emphasize, and I was trying to sort of lay the foundation during the prepared remarks. I would just recognize that in earlier stage, we have nine approvals and four with OS. That basis allows us to drive curative setting combos in a way that is advantageous for the patient and also often uniquely ability for us. The specific study that you're, I think, referring to is the announcement with our partners, Moderna, of going in lung cancer. .

Specifically, that one is built around KEYNOTE-671, which has an OS benefit and is approved. But the interesting thing is in the sort of neoadjuvant or perioperative followed by surgery, there becomes a situation where we can discern who has a pathological complete response and who doesn't have a pathological complete response.

And that patient population that does not have a pathological complete response, there is the need and our ability to provide other combinations in that space, and that is where we're driving the INT in that situation in lung cancer.

Peter Dannenbaum - Merck & Co Inc - Senior Vice President, Investor Relations

Great. Thanks, Mohit, and thank you all for your good questions today. And as always, we're available for follow-ups. Please reach out when you want. Take care. Thank you very much.

Operator

Thank you. This does conclude today's conference. We thank you for your participation. You may disconnect your line at this time.

DISCLAIMER

Refinitiv reserves the right to make changes to documents, content, or other information on this web site without obligation to notify any person of such changes.

In the conference calls upon which Event Transcripts are based, companies may make projections or other forward-looking statements regarding a variety of items. Such forward-looking statements are based upon current expectations and involve risks and uncertainties. Actual results may differ materially from those stated in any forward-looking statement based on a number of important factors and risks, which are more specifically identified in the companies' most recent SEC filings. Although the companies may indicate and believe that the assumptions underlying the forward-looking statements are reasonable, any of the assumptions could prove inaccurate or incorrect and, therefore, there can be no assurance that the results contemplated in the forward-looking statements will be realized.

THE INFORMATION CONTAINED IN EVENT TRANSCRIPTS IS A TEXTUAL REPRESENTATION OF THE APPLICABLE COMPANY'S CONFERENCE CALL AND WHILE EFFORTS ARE MADE TO PROVIDE AN ACCURATE TRANSCRIPTION, THERE MAY BE MATERIAL ERRORS, OMISSIONS, OR INACCURACIES IN THE REPORTING OF THE SUBSTANCE OF THE CONFERENCE CALLS. IN NO WAY DOES REFINITIV OR THE APPLICABLE COMPANY ASSUME ANY RESPONSIBILITY FOR ANY INVESTMENT OR OTHER DECISIONS MADE BASED UPON THE INFORMATION PROVIDED ON THIS WEB SITE OR IN ANY EVENT TRANSCRIPT. USERS ARE ADVISED TO REVIEW THE APPLICABLE COMPANY'S CONFERENCE CALL ITSELF AND THE APPLICABLE COMPANY'S SEC FILINGS BEFORE MAKING ANY INVESTMENT OR OTHER DECISIONS.

©2024, Refinitiv. All Rights Reserved.